

Public Health Emergency Operations Center "COUSP DRC"

MVE-17 Incident Management System

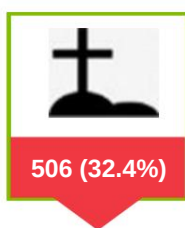
Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°051/MVB_04/07/2026

Provinces affected (3)	ITURI, NORTH KIVU & SOUTH KIVU
Affected Health Zones (36)	- Ituri (24/36): Aru, Aungba, Bambu, Bunia, Damas, Drodro, Gety, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara, Tchomia, Kambala, Nia-Nia, Fataki, Mandima and Lolwa. - North Kivu (11/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha, Masereka, Vuhovi, Mabalako and Musienene. - South Kivu (1/34): Miti-Murhesa
Reporting date	July 4, 2026
Publication date	July 5, 2026



Total confirmed cases
for the 3 provinces



Cumulative deaths
among confirmed cases
and case fatality rate



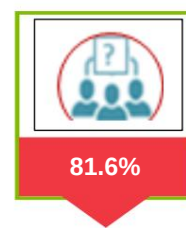
Patients in
isolation -
hospitalization



Cumulative number
of recoveries



Suspected cases of
the day



Contact tracing rates
for the 2 provinces

* Data updated following the DHIS2 database cleaning and harmonization process, including the consolidation of information from health zones, laboratories and case management structures.

Suspicious deaths occurring in the community are subject to investigation.

further investigation with a view to possible reclassification (Non-cases, probable cases)

1. HIGHLIGHTS

Thirty -three (33) new confirmed cases, including 6 community deaths, were reported during the day on July 4, 2026, in the provinces of Ituri (30 cases, including 6 deaths) and North Kivu (3 cases). These confirmed cases come from the health zones of **Rwampara (11), Bunia (7), Nizi (7), Komanda (3), Mongbwalu (2)** in Ituri and **Musienene (2) and Katwa (1)** in North Kivu.

- **Eight (8)** deaths were recorded among patients with Ebola virus disease in Ebola treatment centers in the provinces of Ituri (2 at Ebola Treatment Center Bunia, 2 at Ebola Treatment Center Mongbwalu, 2 at Ebola Treatment Center Elikya and 1 at Ebola Treatment Center Komanda) and North Kivu (1 at Ebola Treatment Center Katwa).
- **Fifteen (15)** patients with Ebola virus disease in Ituri province (4 at CTE Rwampara, 3 at CTE Mongbwalu, 3 at CTE Bunia, 2 at CTE Komanda, 1 at CTE CME, 1 at CTE Elikya and 1 at CTE Nyankunde) have been declared cured after obtaining negative control tests.
- **Start of clinical trials at the CTE CME in the Rwampara health zone in Ituri.**

2. EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

Ituri province, located in the northeast of the Democratic Republic of Congo, shares a long and porous border with Uganda and South Sudan. For over two decades, it has faced a chronic humanitarian crisis linked to armed conflict and recurring population displacements. Its population is estimated at over 8 million, including more than one million internally displaced persons. The Mongbwalu health zone, considered the starting point of the epidemic, is located in the Djugu territory, 70 km from Bunia. This area is characterized by the presence of armed groups, frequent population movements towards Uganda, and numerous artisanal mining sites attracting migrant workers. In September 2018, Ituri province had already been affected by the 10th Ebola Virus Disease (EVD) epidemic.

In North Kivu, the current epidemic, caused by the Bundibugyo ebolavirus, was officially declared on May 15, 2026, by the Minister of Public Health. The three affected provinces have a total population estimated at nearly 15 million inhabitants, with massive internal displacement and cross-border flows to neighboring countries.

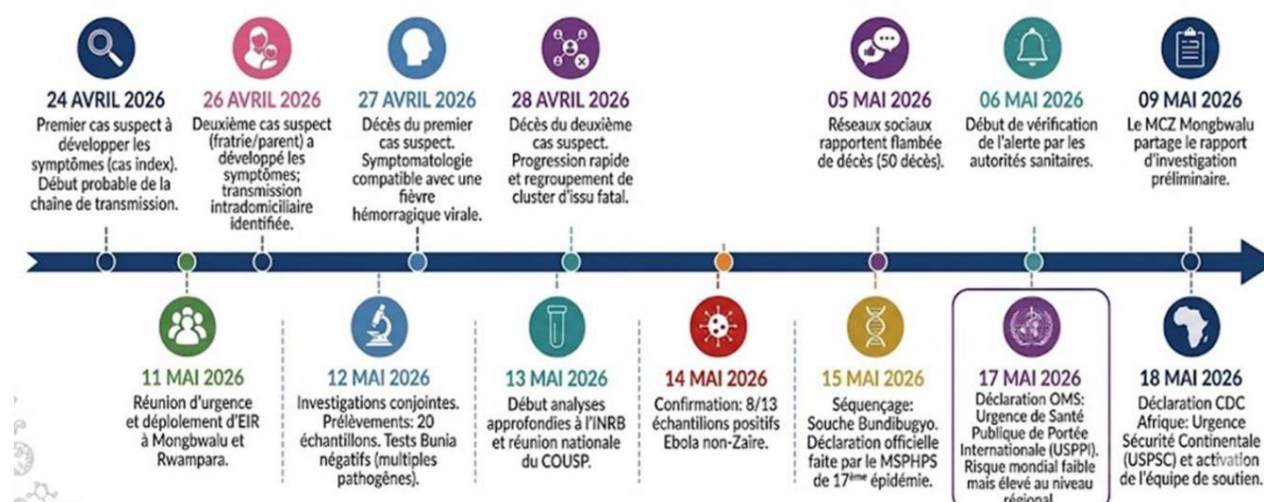


Figure 1. Chronology of events of the Ebola-Bundibugyo virus disease epidemic.

3. DETAILED EPIDEMIOLOGICAL ANALYSIS

3.1 Spatial distribution and active hotspots as of July 4, 2026

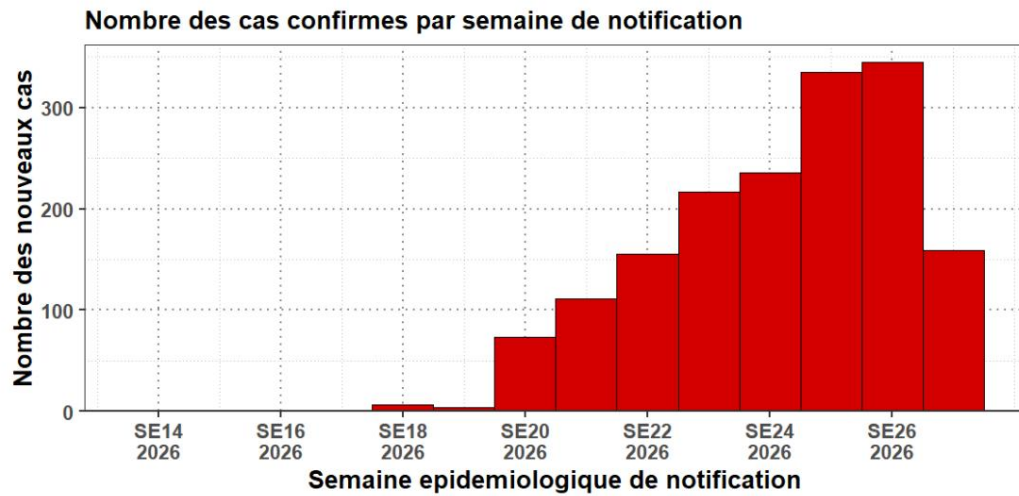
Table 1. Distribution of confirmed cases and deaths by affected province as of July 4, 2026.

Province	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones	New confirmed cases (24 hours)
Ituri	1,417	424*	29.9% 24 out of 36 (66.7%)	57.4% 11 out	30
North Kivu	141	81*	of 34 (32.3%)	33.3% 1 out of 34 (2.9%)	3
South Kivu	3	1	1,561 32.4%	36 out of 104 (34.6%) *7	0
Total	deaths in Ituri and 1 death in North Kivu were recorded among patients with				33

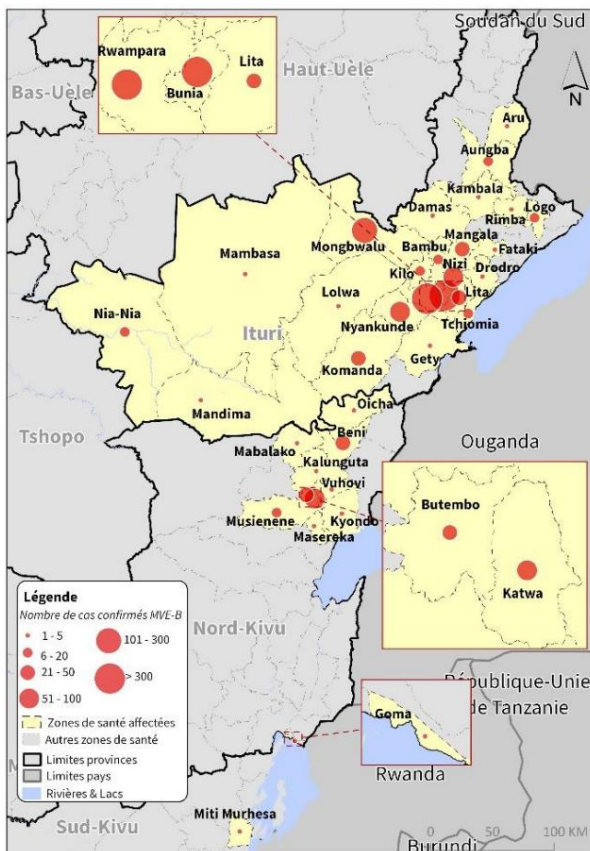
Ebola virus disease (previous confirmed cases) in Ebola treatment centers.

At the end of the day on July 4, 2026, no new health zones were affected. The number of affected health zones remains at 36/104 (section 3.1). Since May 26, 2026, the province of South Kivu has not recorded any confirmed cases, suggesting an absence of recent transmission.

3.3 Time-bound analysis: An increasing number of confirmed cases is observed from one week to the next, reflecting ongoing community transmission of the disease, although the number of cases appeared to be stable during epidemiological weeks 23 and 24. The intensification of public health actions in epidemiological and biological surveillance (decentralization of diagnostic capabilities) contributed to early detection, confirming continued and increased community transmission in week 25. Compared to all the weeks in which cases have been recorded since the beginning of the epidemic, weeks 25 and 26 represent the peak so far, with more than 300 cases per week.



3.4 Epidemic mapping



Figures 2 and 3. Spatial distribution of confirmed Ebola cases by affected health zones and in the three affected provinces as of July 4, 2026.

Table 2. Distribution of confirmed cases and deaths of Ebola virus disease by province and health zone (July 4, 2026)

Province / Health Zone	Cumulative confirmed cases (n)	Cumulative confirmed deaths (n)	Case fatality rate (CFR%)
ITURI	1417	424	29.9
Bunia	438	118	26.9
Rwampara	344	71	20.6
Mongbwalu	278	141	50.7
Nyankunde	95	17	17.9
Nizi	72	19	26.4
Lita	33	12	36.4
Komanda	22	8	36.4
Bamboo	18	5	27.8
Kilo	9	1	11.1
Mangala	26	14	53.8
Tchomia	15	2	13.3
Damascus	5	0	0.0
Aungba	6	2	33.3
Rimba	3	0	0.0
Aru	3		33.3
Mambasa	2		50.0
Logo	7	1	0.0
Drodro	3	1	100.0
Gety		0	0.0
Kambala	1	3	100.0
Nia-Nia	2	0	41.7
Fataki	12 1	2	0.0
Mandima	4		25.0
Lolwa			100.0
Other areas not yet identified	1 17	5 0 1 1 0	0.0
NORTH KIVU	141	81	57.4
Butembo	40	18	45.0
Katwa	52	38	73.1
Blessed	29	17	58.6
Oicha	3	2	66.7
Kalunguta	2		50.0
Kyondo	4	1	50.0
Goma		2	0.0
Masereka	1	0	0.0
Vuhovi			100.0
Mabalako		0	0.0
Musienene	2	1	33.3
SOUTH KIVU	1 1 6 3	0 2 1	33.3
Miti-Murhesa	3	1	33.3
TOTAL	1,561	506	32.4

Over the past 24 hours, **33 new confirmed cases and 14 deaths have been recorded** in the provinces of Ituri (30 cases and 13 deaths) and North Kivu (3 cases and 1 death), highlighting the progression of the epidemiological situation of the 17th Ebola Virus Disease (EVD) epidemic in the Democratic Republic of Congo. Thus, the cumulative total stands at 1,561 confirmed cases and 506 deaths, expressing an overall case fatality rate of 32.4%.

Provincial dynamics and case concentration

The **Ituri** province alone accounts for **90.8%** of cases (n=1,417) and **83.5%** of deaths (n=424), representing a case fatality rate of **29.9%**, making it the absolute epicenter of the epidemic. The health zones of Bunia (438 cases), Rwampara (344 cases), and Mongbwalu (278 cases) remain the most affected, accounting for approximately 74.8% of reported cases at the provincial level and 67.9% at the national level. Within these same health zones, moderate case fatality rates were observed in Rwampara (20.6%) and Bunia (26.9%), while the Mongbwalu zone **also** has a high case fatality rate of **50.7%** (141 deaths out of 278 cases), suggesting persistent challenges in early intervention and access to care. It should be noted that 17 additional cases (with no associated deaths) have been reported while the health zones of origin remain to be identified, which could indicate a geographical spread requiring further investigation.

Severity of the epidemic in North Kivu

A total of 141 confirmed cases and 81 deaths have been reported in North Kivu province since the start of the epidemic, representing a **case fatality rate of 57.4%**. This province presents the most concerning profile in terms of severity, with a case fatality rate significantly higher than the national average. This high case fatality rate is concentrated in the health zones of **Katwa** (73.1%), **Oicha** (66.7%), **Beni** (57.1%), **Kalunguta**, **Kyondo**, and **Musienene** (50.0%) respectively. The Katwa health zone has the highest number of reported cases (n=52), followed by the Butembo health zone (n=40) with a case fatality rate of 45.0%. A case fatality rate of 100% was exceptionally observed in the Vuhovi health zone (which has a small population).

4. RESPONSE ACTIONS BY PILLAR

4.1 Coordination

National / Ituri & Kinshasa

- Holding the coordination meeting with the response team and technical partners and financial support for field activities.

National level :

- Coordination meeting with an agenda: 100%
- Action items completed for daily coordination meetings: 80%

Provincial level :

- Coordination meeting with an agenda: 100% for ITURI and North Kivu; South Kivu: ND
- Action items completed for daily coordination meetings: N/A for the three provinces (ITURI, North Kivu and South Kivu)

Zonal Level :

- Coordination meeting with an agenda: N/A
- Action items completed for daily coordination meetings: N/A

4.2 Epidemiological surveillance

Table 3. Management of epidemiological alerts (24h) as of July 4, 2026

Indicator	Ituri	North Kivu	South Kivu	Total
Report (D-1)				15
New Alerts		0	2	1,229
Total alerts for the day		778	43	1,244
Alerts investigated		778		1002
Investigation rate		778	45 2	80.5%
Alerts confirmed	Alive		4.4% 2	244
	Deceased	13,408,421,222 52.7% 199 100.0% 43 34	0	110
Total suspected cases for today	275	77	2	354

During the day of July 4, **17 surveillance reports** were received, including **13 in Ituri**, **4 in South Kivu** and **none in North Kivu**.

In total, **1,229 new alerts** were notified, bringing the total number of alerts recorded during the day to **1,244**. The majority of alerts came from **North Kivu (778; 63.3%)**, followed by **Ituri (408; 33.2%)** and **South Kivu (43; 3.5%)**.

Of the **1,224 alerts recorded**, **1,002 (81.8%)** were investigated. **North Kivu** achieved a **100% investigation rate**. South Kivu province recorded the lowest investigation rate at 4.4%.

Of the confirmed living suspected cases, **81.6% (199/244)** were from Ituri, **17.6% (43/244)** from North Kivu, and **0.8% (2/244)** from South Kivu. Regarding community deaths, **69.1% (76/110)** were reported in Ituri and **30.9% (34/110)** in North Kivu, while no suspected deaths were recorded in South Kivu.

Table 4. Contact tracing of confirmed cases as of July 4, 2026

Province	Contacts under follow-up (n)	Contacts viewed (n)	Follow-up rate (%)
Ituri*	7,467	6,148	82.3%
North Kivu	2,612	2,073	79.4%
Total	10,079	8,221	81.6%

* The follow-up rate was calculated only for health zones that submitted reports (completeness: 90%). The health zones of Bunia, Aungba, and Nia-Nia (Ituri) did not have data available at the time of report consolidation.

On July 4, **10,079** contacts were under follow-up in Ituri and North Kivu provinces, of which **8,221 (81.6%)** were actually seen during the day. Ituri had the largest number of contacts under follow-up (7,467; 74.1% of the total), with 6,148 contacts seen, corresponding to a follow-up rate of 82.3%. In North Kivu, 2,612 contacts were under follow-up, of which 2,073 (79.36%) were visited. The overall rate of 81.6% remains below the recommended operational target for an effective Ebola virus disease response (≥ 90%). These results underscore the need to strengthen contact tracing and follow-up activities, particularly in Ituri, to improve the completeness of daily monitoring and reduce the risk of secondary transmission within the community.

Update on checkpoint and point of entry (PoC/PoE) activities

- **Ituri** : Nine (9) alerts were notified to the PoE/PoC, including seven (7) confirmed live cases and two (2) intercepted bodies. The alerts were reported to the Mabakese, Foner Mambasa, Mudzibala, and Iga-Barrière PoCs. The confirmed live cases were referred to care facilities, while the bodies were taken to health facilities for collection and safe handling.
- **North Kivu**: Three (3) live alerts were notified to the Kasitu and Mukulia PoCs. After thorough investigation, one (1) alert was maintained and referred, while two (2) were invalidated. Co-facilitation of population mobility mapping continued in the Lubero territory.

Table 5. Monitoring of indicators at Points of Entry and Points of Control (PoE/PoC), as of July 4, 2026

INDICATORS	Ituri	North Kivu	Overall
Proportion of PoC enabled and PoE enhanced	98.1% (54/55)	100.0% (14/14)	98.5% (68/69)
Number of people who went through PoE/PoC testing,	91,610	62,320	153,930
% of travelers screened, % of	94.2%	96.8%	95.2%
travelers who washed their hands, % of	95.1%	96.8%	95.8%
travelers made aware of the issue	97.6%	96.8%	97.3%
Number of alerts notified	9	3	12
Number of validated live alerts	7	3	10
Number of lifeless bodies intercepted today	2	0	2
Number of problematic contacts intercepted today; %	0	0	0
of validated live alerts referred to CT/CTE; % of	100%	33.3%	80.0%
deceased bodies swabed	100.0%	NA	100.0%
Number of positive results of referred suspected cases	0	0	0

4.3. Laboratory

- **Ituri** : 99 samples collected and analyzed (**positivity 30.3%; n=30**).
- **North Kivu** : 73 samples collected and analyzed (**positivity 4.1%; n=3**).
- **South Kivu** : 2 samples collected and ongoing analysis.

4.4. Infection Prevention and Control (IPC)

North Kivu: Thirty (30) death reports were carried out in response to 29 death alerts; 28 bodies were swabed and secured, one body was not swabed, and six bodies were reported. The briefing for the Beni Health Zone (ZS) death report teams has begun. The IPC team was attacked and its vehicle stoned during the decontamination of the Kiharo Medical Center in Rughendia District.

4.5 Holistic care

- **Seven (8)** deaths were recorded among patients with Ebola virus disease in Ebola treatment centers in the provinces of Ituri (2 at Ebola Treatment Center Bunia, 2 at Ebola Treatment Center Mongwalu, 2 at Ebola Treatment Center Elikya and 1 at Ebola Treatment Center Komanda) and North Kivu (1 at Ebola Treatment Center Katwa).

- **Fifteen (15)** patients with Ebola virus disease in Ituri province (4 at CTE Rwampara, 3 at CTE Mongbwalu, 3 at CTE Bunia, 2 at CTE Komanda, 1 at CTE CME, 1 at CTE Elikya and 1 at CTE Nyankunde) have been declared cured after obtaining negative control tests.
- By the end of the day on July 4th, 778 hot meals had been served, including 138 to confirmed cases, 133 to suspected cases, 454 to people with disabilities, and 53 to caregivers. A premature baby weighing 2,300 g was born at the Rwampara Ebola Treatment Center, where a woman 32 weeks pregnant was also admitted.

North Kivu:

- Official opening of the Beni CTE with a capacity of 20 beds; •
- Transfer of a confirmed case (PPL) from the Musienene CT to the Katwa CTE.

Table 6. Analysis of community deaths and hospital mortality during the Ebola Bundibugyo virus disease epidemic in the DRC as of July 4, 2026

Province	Zone de sante	Cas confirmés	Deces confirmés - statut final	Taux de létalité global (communautaire+Intra Hosp)	Deces a la notification	Cas confirmés notifiés dans 24h	Deces confirmés parmi les notifiés dans 24h	Taux de létalité parmi les cas notifiés dans le 24h de leur DDS	Deces a la notification OU notifiés dans 24h	Proportion des décès communautaire	Cas confirmés notifiés après 24h	Deces confirmés parmi les notifiés après 24h	Taux de létalité parmi les cas notifiés après 24h
Ituri	Bunia	465	90	19.4%	88	109	20	18.3%	89	98.9%	307	60	19.5%
Ituri	Rwampara	362	67	18.5%	49	111	25	22.5%	55	82.1%	199	30	15.1%
Ituri	Mongbwalu	283	105	37.1%	105	66	27	40.9%	105	100.0%	204	72	35.3%
Ituri	Nizi	110	33	30.0%	32	21	9	42.9%	32	97.0%	61	13	21.3%
Ituri	Nyankunde	100	7	7.0%	6	38	2	5.3%	6	85.7%	59	5	8.5%
Ituri	Lita	45	17	37.8%	17	11	4	36.4%	17	100.0%	25	9	36.0%
Ituri	Mangala	35	15	42.9%	15	9	4	44.4%	15	100.0%	7	1	14.3%
Ituri	Bambu	27	6	22.2%	6	7	0	0.0%	6	100.0%	14	3	21.4%
Ituri	Komanda	22	6	27.3%	6	6	1	16.7%	6	100.0%	11	4	36.4%
Ituri	Tchomia	18	7	38.9%	4	4	1	25.0%	4	57.1%	13	6	46.2%
Ituri	Kilo	12	2	16.7%	2	3	0	0.0%	2	100.0%	7	2	28.6%
Ituri	Nia Nia	9	3	33.3%	3	2	0	0.0%	3	100.0%	5	2	40.0%
Ituri	Damas	7	2	28.6%	2	1	0	0.0%	2	100.0%	4	1	25.0%
Ituri	Aungba	5	2	40.0%	2	3	1	33.3%	2	100.0%	1	0	0.0%
Ituri	Mambasa	5	3	60.0%	3	1	0	0.0%	3	100.0%	1	0	0.0%
Ituri	Aru	4	1	25.0%	1	2	1	50.0%	1	100.0%	1	0	0.0%
Ituri	Mandima	4	1	25.0%	1	0	0	0.0%	1	100.0%	3	1	33.3%
Ituri	Drodoro	3	3	100.0%	3	0	0	0.0%	3	100.0%	3	3	100.0%
Ituri	Rimba	3	0	0.0%	0	0	0	0.0%	0	0.0%	3	0	0.0%
Ituri	Logo	2	0	0.0%	0	1	0	0.0%	0	0.0%	1	0	0.0%
Ituri	Fataki	1	0	0.0%	0	0	0	0.0%	0	0.0%	1	0	0.0%
Ituri	Gely	1	0	0.0%	0	1	0	0.0%	0	0.0%	0	0	0.0%
Ituri	Kambala	1	0	0.0%	0	0	0	0.0%	0	0.0%	0	0	0.0%
Ituri	Lolwa	1	1	100.0%	1	0	0	0.0%	1	100.0%	1	1	100.0%
Nord Kivu	Katwa	55	28	50.9%	28	45	25	55.6%	28	100.0%	6	2	33.3%
Nord Kivu	Beni	28	17	60.7%	13	5	4	80.0%	13	76.5%	17	9	52.9%
Nord Kivu	Bulembu	24	11	45.8%	9	6	2	33.3%	10	90.5%	18	9	50.0%
Nord Kivu	Kalunguta	2	1	50.0%	0	1	0	0.0%	0	0.0%	0	0	0.0%
Nord Kivu	Kyondo	2	1	50.0%	0	0	0	0.0%	0	0.0%	2	1	50.0%
Nord Kivu	Goma	1	0	0.0%	0	1	0	0.0%	0	0.0%	0	0	0.0%
Nord Kivu	Mabalako	1	0	0.0%	0	0	0	0.0%	0	0.0%	1	0	0.0%
Sud Kivu	Mt Murhesa	3	1	33.3%	1	0	0	0.0%	1	100.0%	1	1	100.0%
TOTAL	TOTAL	1,641	430	26.2%	397	454	126	27.8%	405	94.2%	976	235	24.1%

The epidemic shows an overall lethality rate , % but highly marked in Ituri especially in ZS in Mongbwalu with the highest number of deaths (=). In contrast, increased virulence is observed in North Kivu in the Katwa health zone with a lethality rate of %.

The epidemic profile is marked , % of deaths occurring in the community (92.3%), which demonstrates by invisible active transmission and a collapse in early case detection, highlighting shortcomings in rapid access to care.

Mental health activities and psychosocial support

Table 7. Key indicators of mental health and psychosocial support activities as of July 4, 2026

KEY INDICATORS	ITURI	NORTH KIVU	SOUTH KIVU
New confirmed cases admitted to the CTE who benefited from SMSPS interventions	10 (52.6%)	2 (100.0%)	0
Confirmed cases currently being monitored that have received interventions SMSPS	33 (34.4%)	7 (100.0%)	0
New suspected cases that benefited from SMSPS interventions: 8 (27.6%)		7 (100.0%)	0
Suspected cases under follow-up that have received interventions SMSPS	55 (65.5%)	14 (100.0%)	1 (100.0%)
Laboratory results have been announced.	27	19	0
Including positive ones	11	0	0
Number of children separated in daycare who benefited from interventions SMSPS	9	0	0
Number of children separated in the community who benefited from SMSPS interventions	15	0	13
Number of accompanying persons at the CTE who benefited from interventions SMSPS	48	22	0
Number of PPLs who benefited from SMSPS interventions	1	7	0
Number of household and community members who benefited from SMSPS interventions	16	4	0
Number of EDS supported	12	14	0

4.6 Continuity of care

- In total, **109 PPLs** are confirmed, including 34 deaths (31.2%) and **46 recoveries** in the province of Ituri.

4.7 Risk communication and community engagement

Ituri: 2 advocacy meetings were held with community and cultural leaders to solicit their involvement and support in managing resistance and acceptance of response interventions in the Health Zones. 9,934 people were reached by awareness messages delivered by the RECOs in 1,332 households in 7/22 Health Zones (BUNIA, NYAKUNDE, KILO, RWAMPARA, NIZI, ARU, AUNGBWA).

To date, communities are beginning to implement handwashing without any problems; 7,242 people have been reached with awareness-raising topics during educational talks held by community actors in 4 health zones (BUNIA, ARU, NYAKUNDE, AUNGBWA); 2,266 people, including village chiefs and leaders of women's associations, have been engaged in response activities following community dialogue sessions held in the 1 health zone; training of 80 community leaders, Recos, IT and ITAS with support from the government, Cartas and FHI30 in the NaiNia, Bunia and Nizi health zones; 15 broadcasts of public service announcements and messages on EVD preventive measures (community definition of cases, contact tracing form, contact tracing form, and home visit form); 18 broadcasts of public service announcements and messages on EVD preventive measures.

Community feedback: 148 pieces of feedback and informational messages were collected, of which 32 were clarified, representing approximately 21%. The main concerns related to the saturation of treatment centers, delays in emergency departments, patient nutrition, and handwashing facilities.

Support for the pillars of the response

Surveillance: Support for listing 27 people around 2 positive cases including 13 in Shari (5M and 8F) and 14 in Tchunga (4M and 10F); Support for the investigation and pre-listing of 62 CTE RW contacts CREC exchange meeting, Surveillance and 41 Recos (4M & 37F) from Hoho on their availability and their work in the field (contact tracing).

4.8. Logistics

Ituri

- Delivery of a resuscitation kit to the CTE CME and supply of this structure with medicines.
- Delivery of PCI, PEC and monitoring inputs to the Nyankunde and Rwampara SZs, as well as PCI inputs to the Safety and Security pillar.
- Receipt of a cargo of PCI inputs, monitoring and laboratory reagents supplied by the WHO.
- Supply of operations with 60 liters of fuel.
- Continued infrastructure work: expansion of the Mongbwalu CTE by at least 50 beds, construction of 60 additional beds at the Bunia General Referral Hospital and development of the Elikya Treatment Center.

4.9. Security

- The security situation remains calm but unpredictable in Bunia and surrounding areas.

4.10. PSEA (Prevention of Sexual Exploitation and Abuse)

Ituri : Stakeholder briefing and community awareness activities on preventing sexual exploitation and abuse continued in the health zones of Bunia, Rwampara, Mongbwalu, and Aru. No updated figures were reported in the consolidated presentation of July 4, 2026.

5. MAIN CHALLENGES, IMPACTS AND REQUIRED ACTIONS

Challenge	Impact	Actions required
No. 1 Reluctance to undergo post-mortem sampling (EDS/swabs) and community resistance	Confirmation delays, underestimation of cases, disruption of key activities	Intensify CREC, involve religious and community leaders, and strengthen team security.
2 Insufficient capacity of CTEs and saturation in North Kivu (138.8%)	Admission delays, increased risk of nosocomial transmission	Accelerate the construction and expansion of CTEs, deploy additional temporary structures
3 Contact tracking performance not optimal (81.3% vs target 95%)	Uninterrupted transmission chains	Train, deploy and motivate community liaisons (RECOs), enhanced supervision
4 Low reporting of alerts in some affected areas	Undetected transmission, silent spread of the epidemic	Strengthen community surveillance, improve the rapid escalation and investigation of alerts
5 confirmed cases not yet ventilated by health zone (17 cases)	Major gaps in the reconstruction of transmission chains	Conduct a data reconciliation, prune any possible duplicates, and reclassify or clarify the health zone and status (deceased, recovered, active, etc.) Active field verification
6 Insufficient supply of MEG and medical inputs	Weakening of overall care	Advocacy and urgent supply of essential medicines
7 Insufficient PCI inputs (PPE, chlorine) and limited training	High risk of nosocomial infections (96 HCW cases)	Urgent supply, strengthening of IPC training

		including 19 deaths, case fatality rate 19.7%)	
8	Stock of samples not yet analyzed (backlog) at the laboratory North Kivu and late diagnosis	Delayed confirmation, high lethality (54.2%)	Deploy a laboratory catch-up plan, strengthen diagnostic capabilities
9	Insufficient number of ambulances and isolation facilities (gap of 20 centers)	Transfer delays and isolation increase the risk of transmission	Urgent deployment of additional logistical resources and infrastructure
10	Insufficient and weak coordination use of COUSP frames	Operational inefficiency and delayed decisions	Strengthen coordination mechanisms at all levels, and make provincial COUSPs operational.
11.	Insufficient resources financial (gap of 20 million USD)	Limitation of the implementation of all pillars of the response	Urgent mobilization of resources, advocacy with partners
12	Insecurity and limited access (CODECO, ADF)	Restrictions on operations, limited access to high-risk areas	Strengthening security coordination and humanitarian access
13	Continuity of essential services (CEHS) compromised	Increase in indirect mortality (not Ebola)	Effective implementation of free healthcare, strengthening of CEHS
14.	High population mobility	High risk of rapid geographic expansion	Strengthening surveillance at PoE/PoC sites and cross-border coordination

6. PHOTOS OF FIELD ACTIVITIES

Additional CTEs are being built to accommodate at least 450 beds.



**POUR TOUTE INFORMATION
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Capacity building for RECOs on Ebola Virus Disease in Bunia

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**Avec l'appui technique de l'Organisation Mondiale
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